

CONFIDENTIAL MEDICAL RECORD

PATIENT INFORMATION

Field Details

Patient Name James Alan Whitfield

Date of Birth March 14, 1981

Age 44

Sex Male

MRN 00847261

SSN (Last 4) XXXX

Address 412 Cedarwood Lane, Springfield, IL 62704 **Phone** (217) 555-0194

Emergency Contact Sandra Whitfield (spouse) – (217) 555-0217 **Insurance** BlueCross

BlueShield of Illinois – Policy #BCB-7741092 **Primary Care Physician** Dr. Patricia

Nguyen, MD – Westside Family Medicine

INCIDENT SUMMARY

Date of Injury: October 3, 2024 **Location of Incident:** Intersection of Route 36 and Elm Street, Springfield, IL **Mechanism of Injury:** Motor vehicle collision. Patient was a restrained driver of a 2019 Honda Accord traveling westbound when struck on the driver's side by a northbound 2021 Ford F-150 that ran a red light at approximately 45 mph. Patient's vehicle sustained significant lateral deformation. Airbags deployed. Patient was ambulatory at the scene but reported immediate neck pain, left shoulder pain, headache, and dizziness. Emergency Medical Services (EMS) responded within approximately 8 minutes. Patient was transported by ambulance to Memorial Medical Center Emergency Department.

EMERGENCY DEPARTMENT VISIT

Facility: Memorial Medical Center – Emergency Department **Date:** October 3, 2024 **Time of Arrival:** 3:47 PM **Attending Physician:** Dr. Marcus Ellis, MD (Emergency Medicine)

Chief Complaint

Neck pain, left shoulder pain, headache, and dizziness following motor vehicle collision.

Triage Vitals

BP: 148/92 mmHg (elevated, likely pain/anxiety related)

HR: 102 bpm

RR: 18 breaths/min

SpO₂: 98% on room air

Temp: 98.6°F

GCS: 15 (Alert and oriented x4)

Pain Score: 8/10

History of Present Illness

Mr. Whitfield is a 44-year-old male presenting after a T-bone motor vehicle collision. He was the belted driver and reports the airbag deployed. He denies loss of consciousness but notes a brief period of confusion immediately after impact. He complains of severe neck pain radiating into the left shoulder and upper arm, a pounding frontal headache, ringing in both ears (bilateral tinnitus), and blurred vision. He also reports left-sided chest wall pain consistent with seatbelt contusion. He denies shortness of breath, abdominal pain, or lower extremity pain at this time.

Past Medical History

Hypertension (well-controlled on lisinopril 10 mg daily)

Mild hypercholesterolemia (diet-controlled)

No prior spinal or orthopedic surgeries

No prior history of neck or back injuries

Allergies

Penicillin (hives)

Current Medications

Lisinopril 10 mg PO daily

Physical Examination

General: Alert, oriented, anxious male in moderate distress secondary to pain. No acute respiratory distress.

Head/Eyes/Ears/Nose/Throat (HEENT): Normocephalic, atraumatic. Pupils equal, round, and reactive to light bilaterally (3mm → 2mm). No hemotympanum. No periorbital ecchymosis. Mild photophobia noted.

Neck: Midline cervical tenderness to palpation at C4–C6. Paraspinal muscle spasm bilaterally, more pronounced on the left. Range of motion severely limited in all planes secondary to pain. No midline step-offs. Trachea midline.

Chest: Seatbelt ecchymosis across the left chest and shoulder. Tenderness to palpation over the left 4th– 6th ribs anterolaterally. Breath sounds clear and equal bilaterally. No crepitus.

Cardiovascular: Tachycardic, regular rhythm. No murmurs.

Abdomen: Soft, non-tender, non-distended. No guarding or rigidity.

Extremities: Left shoulder tenderness to palpation over the AC joint and rotator cuff region. Decreased active range of motion of the left shoulder secondary to pain. Neurovascularly intact distally. No obvious deformity. Right upper and bilateral lower extremities unremarkable.

Neurological: CN II–XII grossly intact. Strength 5/5 in right upper extremity. Strength 4/5 in left upper extremity (limited by pain). Sensation intact. DTRs 2+ throughout. No Babinski sign.

Diagnostic Studies

Imaging:

Cervical Spine X-Ray (3 views): Alignment maintained. No acute fracture or dislocation identified. Mild straightening of the normal cervical lordosis consistent with paraspinal muscle spasm. Degenerative changes at C5-C6 with mild disc space narrowing.